

A top-down view of a grey desk with various medical and study items: a stethoscope, a laptop displaying a medical website, a tablet, a pair of glasses, a smartphone, a spiral notebook with a pen, and a coffee cup. A large white pill-shaped graphic is centered over the laptop.

Pharm*Achieve*

CANADIAN HEALTHCARE SYSTEM

Qualifying Exam Preparatory Course

COMPETENCIES COVERED IN THIS PRESENTATION...

Providing Care:
Clinical Care

Providing Care:
Distribution

Knowledge
& Expertise

Communication
& Collaboration

Leadership &
Stewardship

Professionalism

LEGEND

ADR	Adverse Drug Reaction	HPFB	Health Products and Food Branch
APAs	Advertising Preclearance Agencies	MDD	Medical Devices Directorate
CDA	Canada's Drug Agency	NHP	Natural Health Product
CABG	Coronary Artery Bypass Graft	NNHPD	Natural and Non-Prescription Health Products Directorate
CDCP	Canada Dental Care Plan	OPA	Ontario Pharmacists Association
CDR	Common Drug Review	OTC	Over-The-Counter
CE	Continuing Education	PCP	Primary Care Provider
CFIA	Canadian Food Inspection Agency	PHAC	Public Health Agency of Canada
CGPA	Canadian Generic Pharmaceutical Association	PMPRB	Patented Medicine Prices Review Board
CHT	Canada Health Transfer	P/T	Provincial and/or Territorial
CIHR	Canadian Institutes of Health Research	RCMP	Royal Canadian Mounted Police
CME	Continuing Medical Education	Rx	Prescription
CPhA	Canadian Pharmacists Association	Rx&D	Canada's Research-Based Pharmaceutical Companies (Now Innovative Medicines Canada)
CPS	Compendium of Pharmaceuticals and Specialities	SAP	Special Access Program
CSHP	Canadian Society of Hospital Pharmacists	TMA	Terms of Market Authorization
CTC	Compendium of Therapeutic Choices		
DIN	Drug Identification Number		
DPD	Drug Product Database		
F&DA	Food and Drugs Act		

HEALTH PORTFOLIO

MINISTRY OF HEALTH

HEALTH PORTFOLIO



HEALTH PORTFOLIO

- ***Health Canada:** Federal department helping Canadians maintain and improve health
- ***Public Health Agency of Canada (PHAC):** Protects the health and safety of all Canadians by focusing on preventing chronic diseases, injuries, and responding to emergencies and infectious outbreaks
- ***Canadian Institutes of Health Research (CIHR):** Major federal funding for health research
- ***Patented Medicine Prices Review Board (PMPRB):** Protects Canadians by ensuring that manufacturers' prices of patented medicines are not excessive
- **Canadian Food Inspection Agency (CFIA):** Mitigate risks of food safety

Health portfolio supports the Minister of Health to maintain and improve health of Canadians

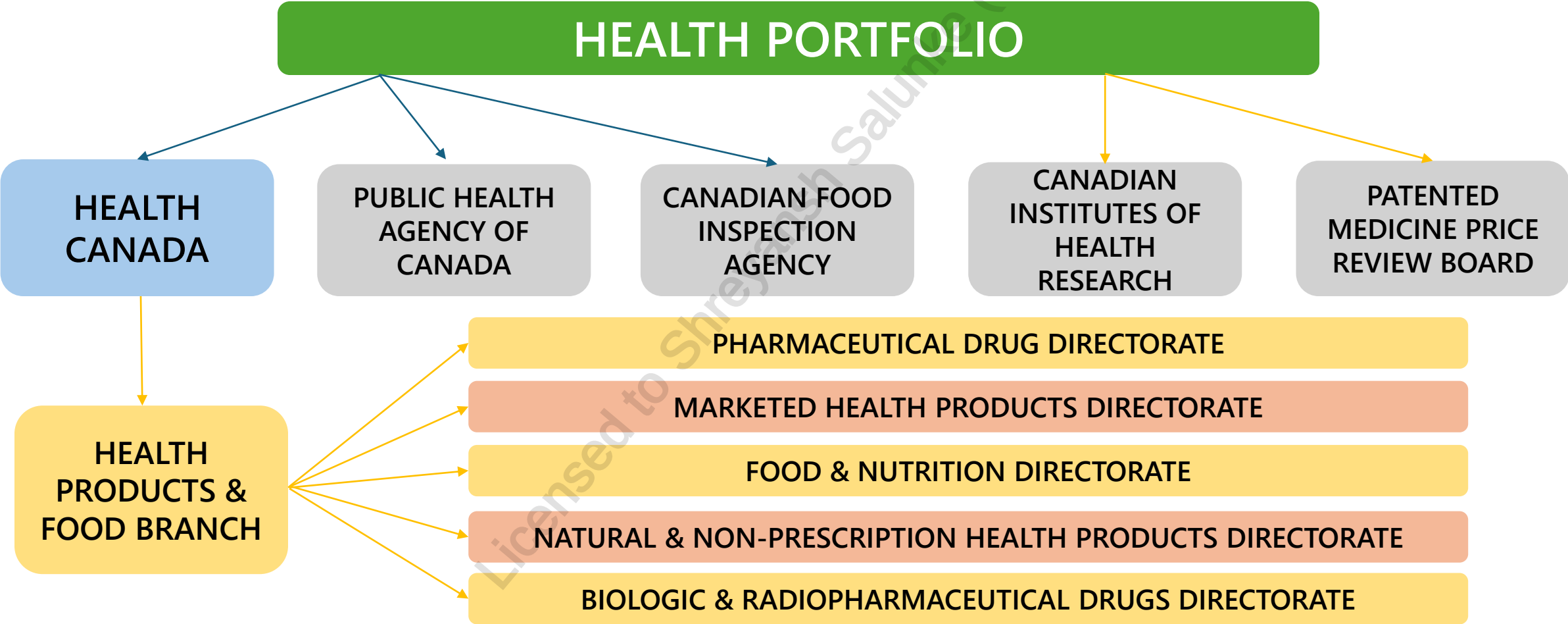
CORE ROLES OF HEALTH CANADA

Federal agency mandated to “promote and protect the health & safety of Canadians”

Partner/leader	Partnering with other federal and P/T bodies to ensure Canadians’ needs are met.
Funder	Provides provincial health care funding through Canada Health Transfer AND <u>direct funding for certain groups</u> (e.g. certain First Nations, Inuit communities, members of the Canadian Forces, RCMP, prison inmates, veterans, some refugees).
Guardian/regulator	Approves drugs, food, NHPs, and medical devices for sale in Canada.
Information provider	Disease surveillance and protection (including recalls)
Service provider	Provides select services in remote areas (e.g. First Nations reserves)

HEALTH PORTFOLIO

MINISTRY OF HEALTH



HEALTH PRODUCTS AND FOOD BRANCH (HPFB)

Branch of Health Canada

Mandate: manage health-related risks and benefits of food and health products

How they fulfill their mandate:

1. Evaluate and monitor efficacy, safety, and quality of health products, foods and veterinary drugs
2. Develop, promote, and implement nutrition and food policies and standards
3. Provide evidence-based and timely information allowing Canadians to make healthy and informed decisions
4. Anticipate and respond to public health and safety issues associated with health products, nutrition, and food

HEALTH PRODUCTS AND FOOD BRANCH (HPFB)

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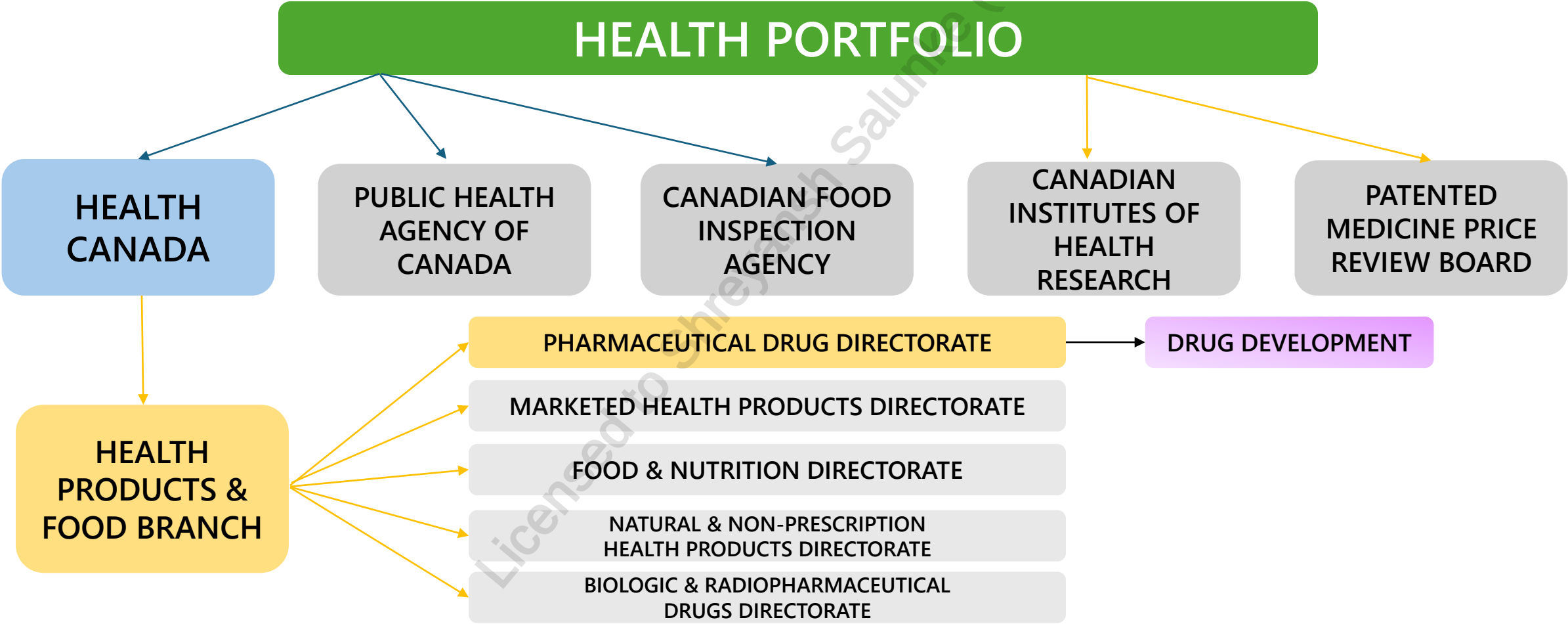
Health
Products
and Food
Branch

- **Pharmaceutical Drugs Directorate (PDD):** Regulator of Rx drugs (during trials) for human use. Verifies that drugs meet **safety, efficacy & quality** requirements of the F&DA
- **Marketed Health Products Directorate:** **Post-marketing surveillance** (safety & effectiveness) data collection and communication (warnings, advisories, recalls) of authorized health products
- **Food & Nutrition Directorate (FND):** Promotes **healthy eating** in Canada; creator of the Canada Food Guide; contributes to initiative to create healthy food environments
- **Natural and Non-prescription Health Products Directorate (NNHPD):** **Authorize the sale** of non-prescription and natural health products in Canada; regulate their safety, efficacy & quality
- **Biologics and Radiopharmaceutical Drugs Directorate (BRDD):** Regulates biologic drugs, radiopharmaceuticals, cells, tissues and organs for human use; also regulates Assisted Human Reproduction
- **Medical Devices Directorate (MDD):** Regulates medical devices for human use

HEALTH PORTFOLIO

MINISTRY OF HEALTH

HEALTH PORTFOLIO



DRUG DEVELOPMENT IN CANADA

Pre-clinical trials

1. Research
2. New substance purification
3. Administration *in vivo* and *in vitro*
4. Further testing and assessments
5. Filing Clinical Trial Application (CTA) to HPFB for approval to begin conducting clinical trials

After HPFB approval:
Clinical trials begin

Phase I

- Purpose: **Safety and dose**
- Participants: 20-100 healthy humans
- Duration: Months

Phase II

- Purpose: **Efficacy and adverse events**
- Participants: 100-300 humans with disease
- Duration: Months-2 years

Phase III

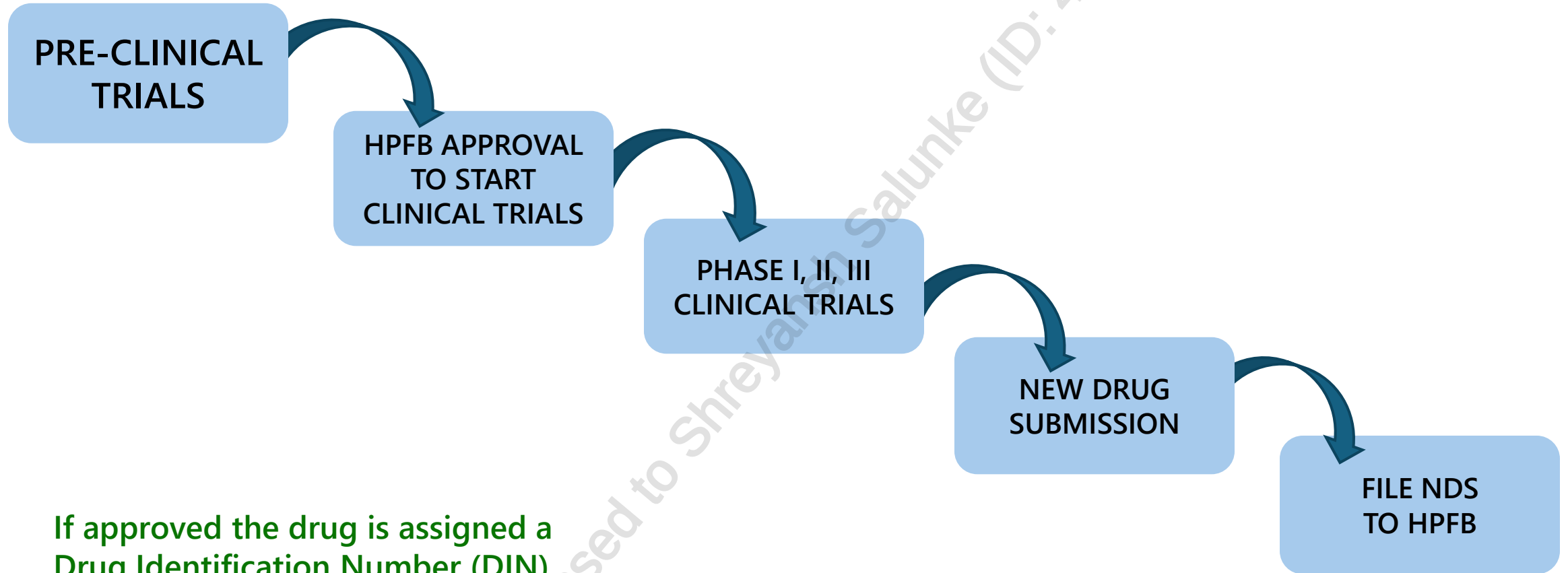
- Purpose: confirm **efficacy**, continue monitoring for **ADRs**, **comparison** to current therapies
- Participants: 300-3000+ humans with disease
- Duration: 1-4 years

Phase IV: Post-marketing surveillance

- Purpose: **long-term benefits and risks, optimal drug use**

If positive results: New Drug Submission is filed to HPFB for approval

DRUG DEVELOPMENT IN CANADA



CHARACTERISTICS OF CLINICAL TRIALS

Phase I, II, or III?

1. Is the treatment better than what's currently available?
2. After 3 years of therapy what side effects would be expected to develop?
3. Is the drug effective?
4. What are the possible side effects?
5. Participants: subjects with disease
6. Participants: healthy subjects
7. What's the highest dose that can be administered safely?

CHARACTERISTICS OF CLINICAL TRIALS

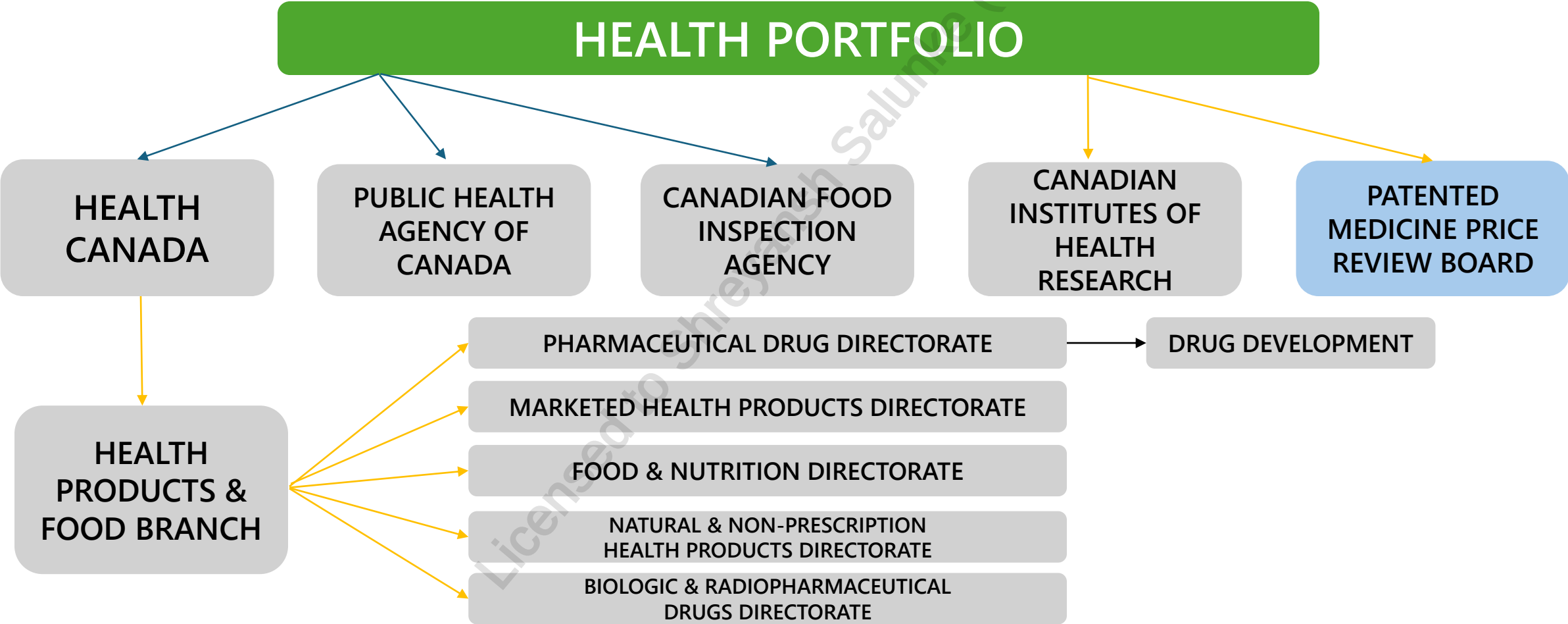
Phase I, II, or III?

1. Is the treatment better than what's currently available? **Phase III**
2. After 3 years of therapy what side effects would be expected to develop? **Phase III**
3. Is the drug effective? **Phase II, III**
4. What are the possible side effects? **Phase II, III**
5. Participants: subjects with disease? **Phase II, III**
6. Participants: healthy subjects? **Phase I**
7. What's the highest dose that can be administered safely? **Phase I**

HEALTH PORTFOLIO

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HEALTH PORTFOLIO



PATENTED MEDICINE PRICES REVIEW BOARD (PMPRB)

Established under the Patent Act: Protects Canadians by ensuring that the prices of patented medicines are not excessive to protect consumer interest.

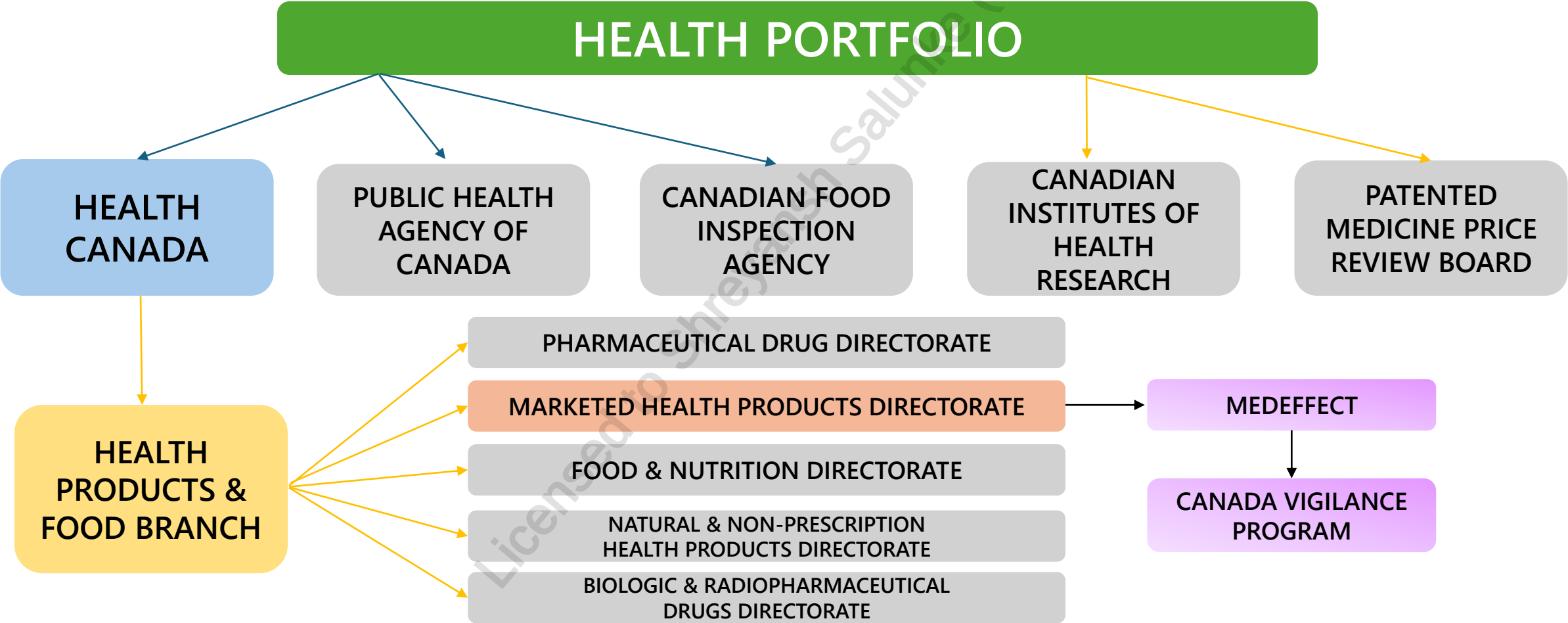
- PMPRB does not have jurisdiction to control non-patented drugs eg. generics

Review Process

- Scientific review conducted to assess level of therapeutic improvement of a patented drug product
- Pricing figures submitted by patent holder are considered
- Each drug is assessed at *DIN level*
- If a drug is found to be excessively priced, manufacturer may voluntarily lower the price (*Voluntary Compliance Undertaking process*)
- If manufacturer refuses, a public hearing is scheduled to decide if the price is too high

HEALTH PORTFOLIO

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MEDEFFECT

Developed by Marketed Health Products Directorate (MHPD)

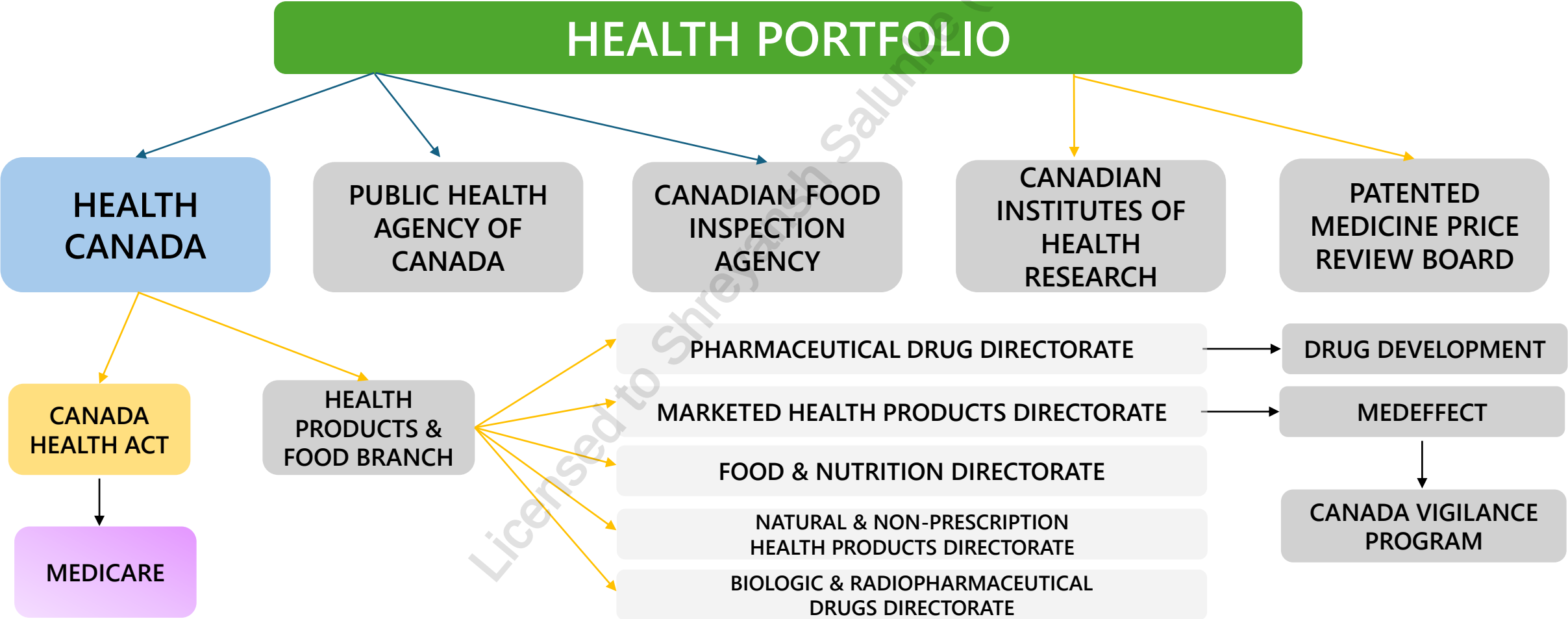
Website where **patients and providers** can obtain information on drug safety alerts, advisories, warnings, and recalls; can be readily accessed by patients and providers

CANADA VIGILANCE PROGRAM Part of MEDEFFECT

- **Post-marketing surveillance:** Collects and analyzes all reports of adverse drug reactions from marketed health products. It also provides a public reporting portal for patients and providers.
- Marketed health products include:
 - Prescription and non-prescription medications (including cannabis and veterinary drugs)
 - Natural health products
 - Biologics (includes biotechnology products, vaccines, fractionated blood products, human blood and blood components, as well as human cells, tissues and organs)
 - Radiopharmaceuticals
 - Disinfectants and sanitizers with disinfectant claims
 - Medical devices

HEALTH PORTFOLIO

MINISTRY OF HEALTH



MEDICARE

- National, publicly-funded (federal, P/T taxation) health insurance program composed of 10 provincial and 3 territorial plans.
- Coverage is provided on basis of medical need, NOT ability to pay
- All residents are offered equal care with access to medically-necessary hospital and physician services

Role of Federal government

- Providing financial support to P/T
- Setting and administering national principles under Canada Health Act
- Directly fund services for certain populations (e.g. prison inmates)
- Health promotion, safety, and regulation (e.g. regulation of pharmaceuticals)
- **NEW:** The National Pharmacare Act (Bill C-64)

P/T Governments

- Administration and delivery of health care services
- Funding comes from federal transfers and provincial treasury, but P/T decide how to allocate spending (e.g. hospitals, specialized clinics)
- Most P/T fund services NOT covered under Canada Health Act for certain groups (e.g. seniors)

CANADA HEALTH ACT



Provides a yearly sum of money called the Canada Health Transfer to the P/T for “medically necessary” physician and hospital services

Primary objective is to protect, promote, and restore the physical and mental well-being of residents of Canada and facilitate reasonable access to health services without financial or other barriers.

- **Public administration:** Administered and operated on non-profit basis by a public authority accountable to P/T government
- **Comprehensiveness:** Insure all medically necessary physician and hospital care services
- **Universality:** Entitle all insured persons to coverage on uniform terms/conditions
- **Accessibility:** Provide all insured persons reasonable access to medically necessary hospital and physician services without financial or other barriers
- **Portability:** Cover all insured persons when relocating to another P/T in Canada and to a limited extent when travelling abroad (P/T only pays same rate paid in Canada, and may require prior approval for non-emergency services delivered outside their jurisdiction)

CANADA HEALTH ACT - MEDICALLY NECESSARY SERVICES

Under the Canada Health Act's "**accessibility**" principle medically necessary services must be fully funded (NO user fees or extra-billing for such services)

What medically necessary services are covered according to the Act?

1. Physician services if deemed medically necessary by the province (some services like doctor's notes, may not be deemed necessary)
2. Hospital care services if deemed necessary by the province (some elective or cosmetic surgeries may not be covered)
3. Anything the province deems medically necessary
4. Prescription drugs are not covered under the Act except through (2) above, i.e. medically necessary "hospital care services"

Are dental services covered by the Canada Health Act?
How about long-term care accommodation?

CANADA DENTAL CARE PLAN (CDCP) – NEW INITIATIVE

- A federal government initiative to cover all or a portion of the dental costs for Canadians
- An individual must submit an application to be approved for coverage under the CDCP

Covered services: FYI

Oral exams and assessments, X-rays, cleanings (scaling and polishing), fluoride treatments and sealants, fillings, root canals, extractions, dentures, some periodontal services, anesthesia, emergency dental services

Qualifications: Must meet all 4 criteria:

1. Be a Canadian resident
2. No access to dental coverage from elsewhere (e.g. through personal or a family member's employment benefits)
3. The individual (and partner if applicable) must have filed tax returns in Canada (in order to assess family income for the previous year)
4. Adjusted family net income is < \$90,000

OTHER TRANSFER PAYMENTS

Equalization Transfer Program

- Addresses fiscal disparities among provinces
- Enables less prosperous provinces to provide services that are comparable to those in other provinces (at comparable levels of taxation)
- Provinces can be net receivers or net contributors depending on tax generation

Territorial Formula Financing

- Annual unconditional transfer that recognizes the high cost of providing public services in the North (Yukon, Northwest Territories, Nunavut)

Canada Social Transfer

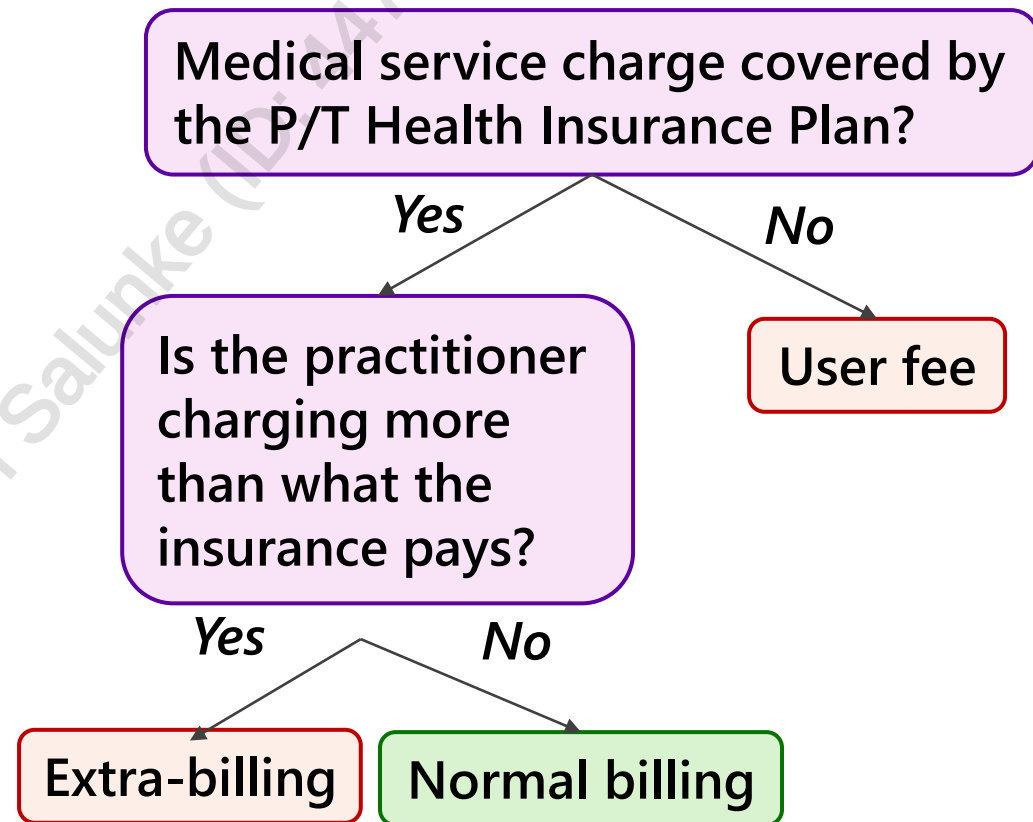
- Third largest federal transfer after CHT and Equalization

Supports three areas for all Canadians:

- Post-secondary education
- Early childhood development, childcare and early learning
- Social assistance and social services

CANADA HEALTH ACT (CHA)- EXTRA-BILLING AND USER FEES

- When the users of a service wholly pay for the service, the fees are called a user fee.
Example: a road toll
- When the users of a service partially pay a fee on top of what is covered by public insurance that is called “extra-billing”
- Under the Canada Health Act, can a province or territory permit a user fee or extra billing for medical services?
 - a. Yes For *non-medically* necessary services
 - b. No For *medically* necessary services



What happens if a province violates the Canada Health Act and charges a user fee or extra-billing for a medically necessary service? Federal Government reduces Canada Health Transfer by a corresponding amount (so province doesn't raise any new money by charging a user fee or allowing practitioners to engage in extra-billing) for that medically necessary service.

SPECIAL ACCESS PROGRAM (SAP)

Allows regulated health care professionals to gain limited access to pharmaceutical products in the following situations:

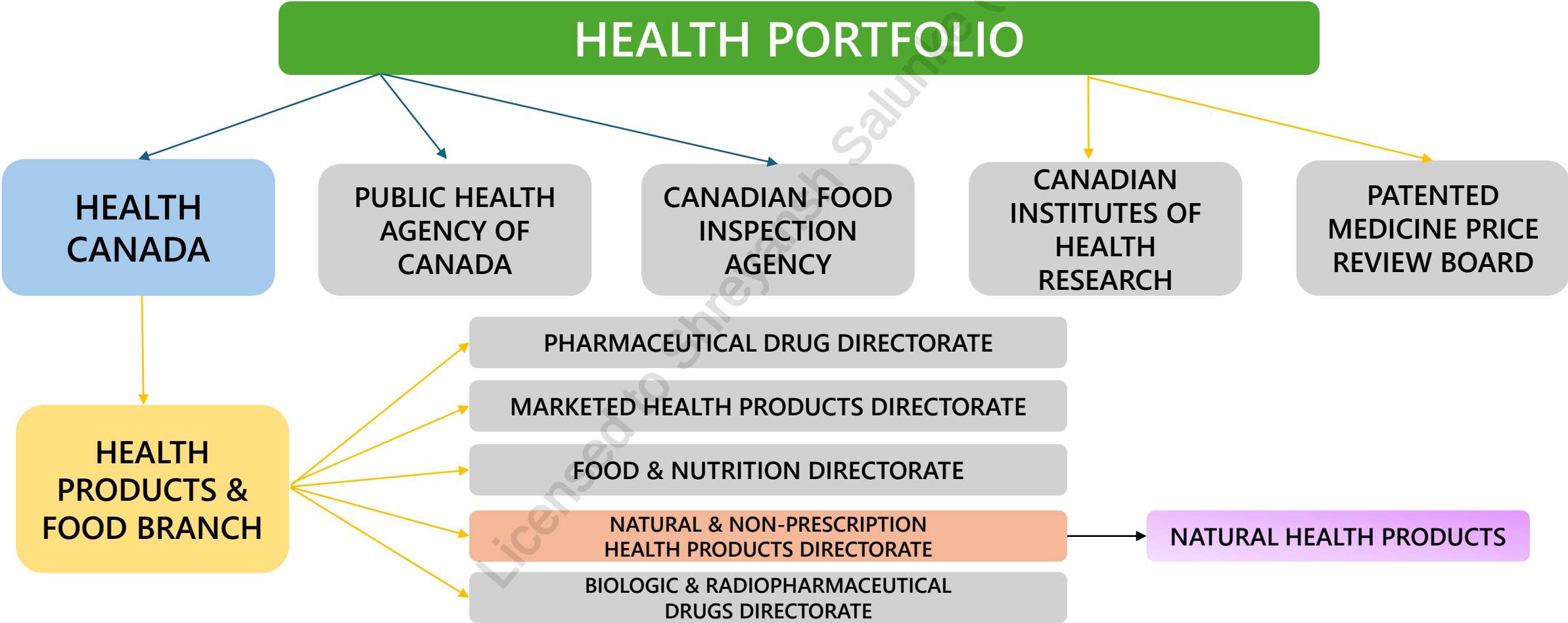
- When the drug is not approved for sale in Canada
- When the drug is requested for emergency use
- When conventional therapies have failed, are unavailable, or are unsuitable to treat a patient
- When a health crisis, such as an outbreak of a communicable disease occurs

Note: a drug that is or contains a restricted drug, as defined in Part J of the F&DA is not eligible for authorization through SAP.

HEALTH PORTFOLIO

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HEALTH PORTFOLIO



NATURAL HEALTH PRODUCTS (NHPS)

Definition and Regulations

A natural health product is defined as:

At least one of the following:

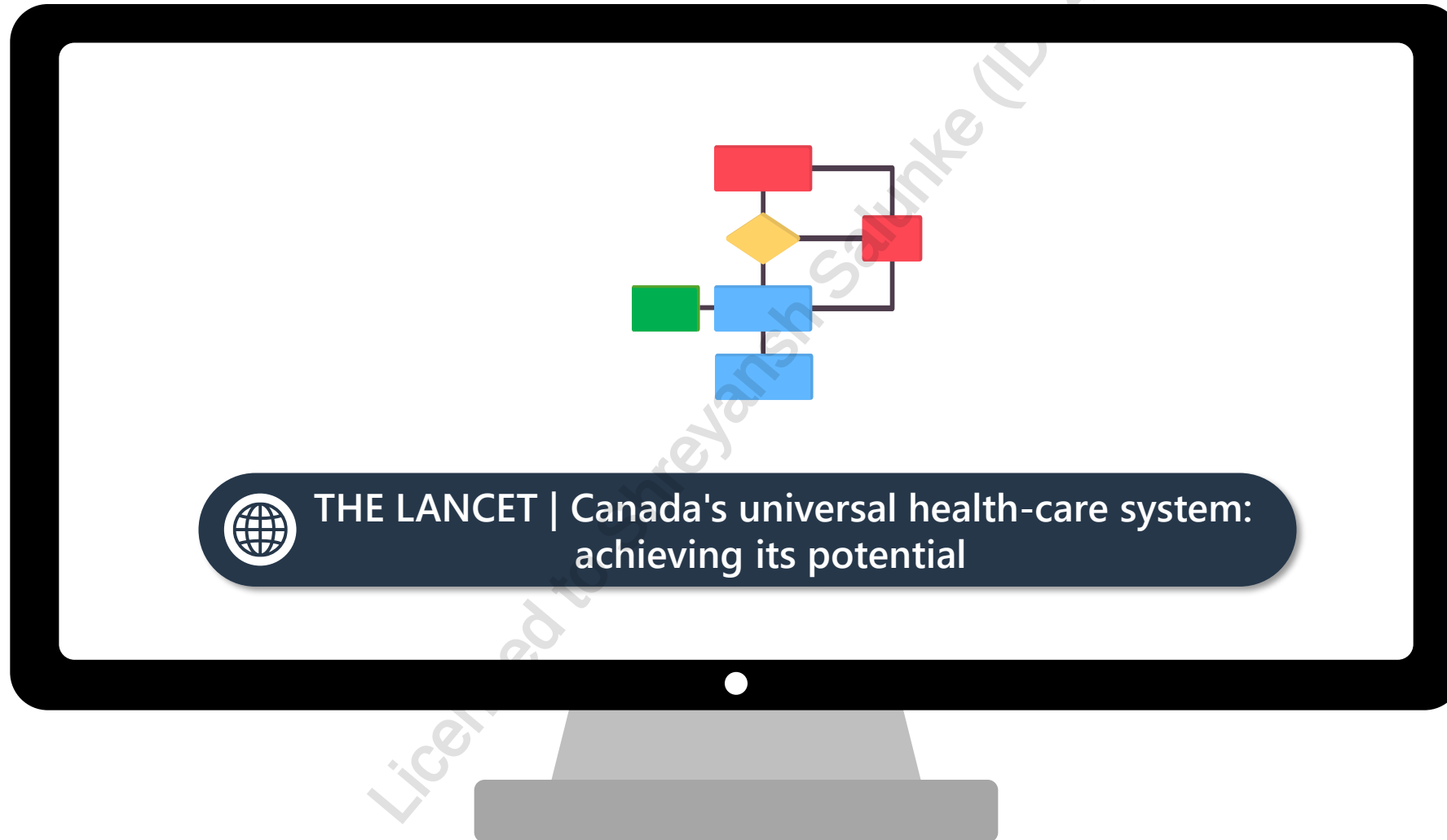
- Vitamins
- Minerals
- Traditional medicines (e.g. Chinese Medicines)
- Homeopathic medicines (*DIN-HM*)
- Herbal remedies
- Probiotics
- Amino acids
- Essential fatty acids

PLUS:

Does NOT require a prescription for sale **AND** is identified by an NPN OR DIN-HM

Regulated By NNHPD:
Regulates sale in Canada by ensuring safety and efficacy.

CANADIAN HEALTHCARE SYSTEM OVERVIEW



LEVELS OF PATIENT CARE



Primary Care

- 1st point of contact
- Primary care provider (PCP) is the first person you normally see (usually a family physician or possibly a nurse practitioner)
- Examples: physicians office, community health centres, nursing stations

Secondary Care

- Specialized care provided by clinicians focusing on a specific area of care, patients are often sent upon referral
- Example: surgical care, rehabilitation, psychiatrists

Tertiary Care

- Academic or specialized health centers providing advanced medical care
- Example: Neonatal ICU, Hospitals with cardiac centers providing angioplasty and Coronary Artery Bypass Graft (CABG) services

CATEGORIES OF CARE

In-patient care

- Hospital stay of >24 hours
- Patient is admitted to the hospital
- Can be within any ward of the hospital

Outpatient care

- Hospital stay of <24 hours
- Patient is NOT admitted to the hospital
- Various clinics within the hospital, such as cardiology, renal, etc.
- May include emergency room visits if patient is not admitted

Ambulatory care

- Ambulate means “to walk”
- Any outpatient care, including primary care, urgent care, and specialty clinic visits

What about accessing care from home? **Telehealth**

- Delivering care through telecommunication – keeps the cost of healthcare down
- Increases access to care in remote areas where there is a shortage of clinicians
- Monitoring chronically ill patients from home and reducing travel and wait times



COMPLEMENTARY AND ALTERNATIVE MEDICINE (CAM)

CAM domains include yoga, relaxation techniques, naturopathy, Ayurveda, Reiki-Qi-Gong, mind-body medicine, chiropractic and osteopathy

Complementary Medicine

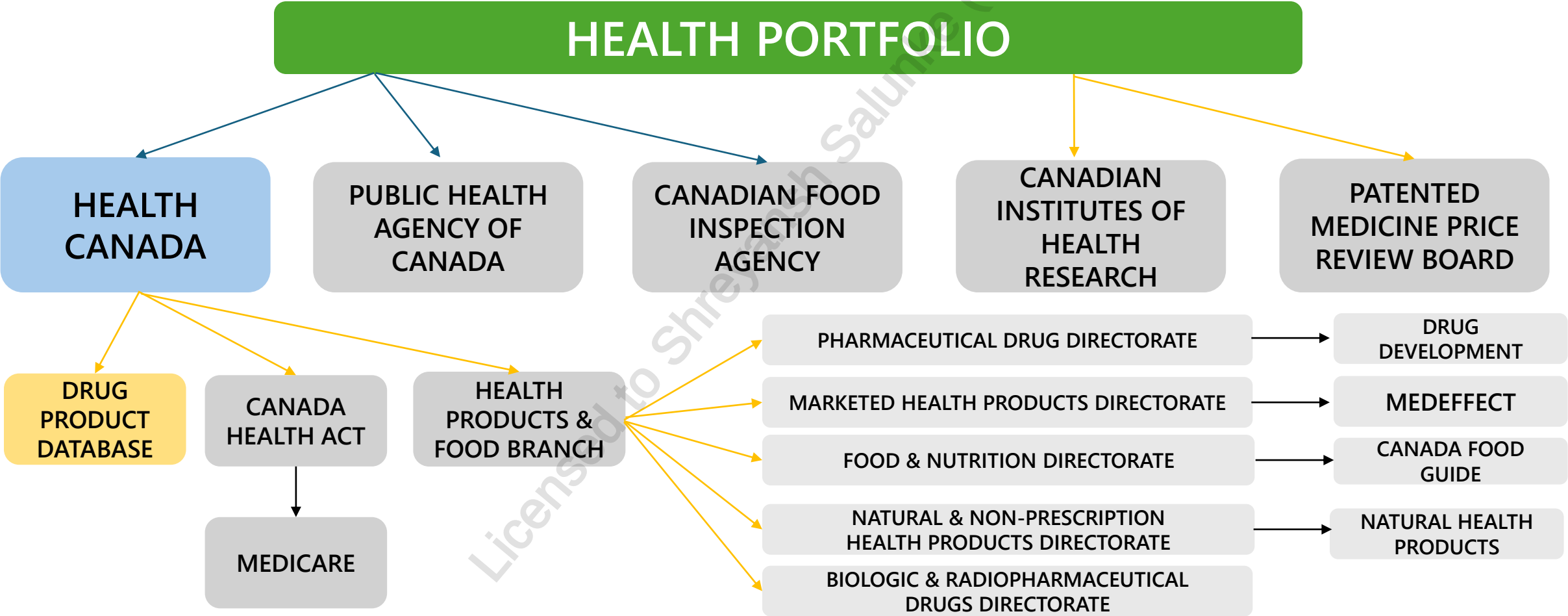
- Treatments used in conjunction with conventional medicine
- Example: A patient with cancer receiving chemotherapy also undergoes acupuncture

Alternative Medicine

- Treatments that are used as a substitute for conventional treatments
- Example: A patient with cancer who declines chemotherapy and manages cancer with herbal medications

HEALTH PORTFOLIO

MINISTRY OF HEALTH

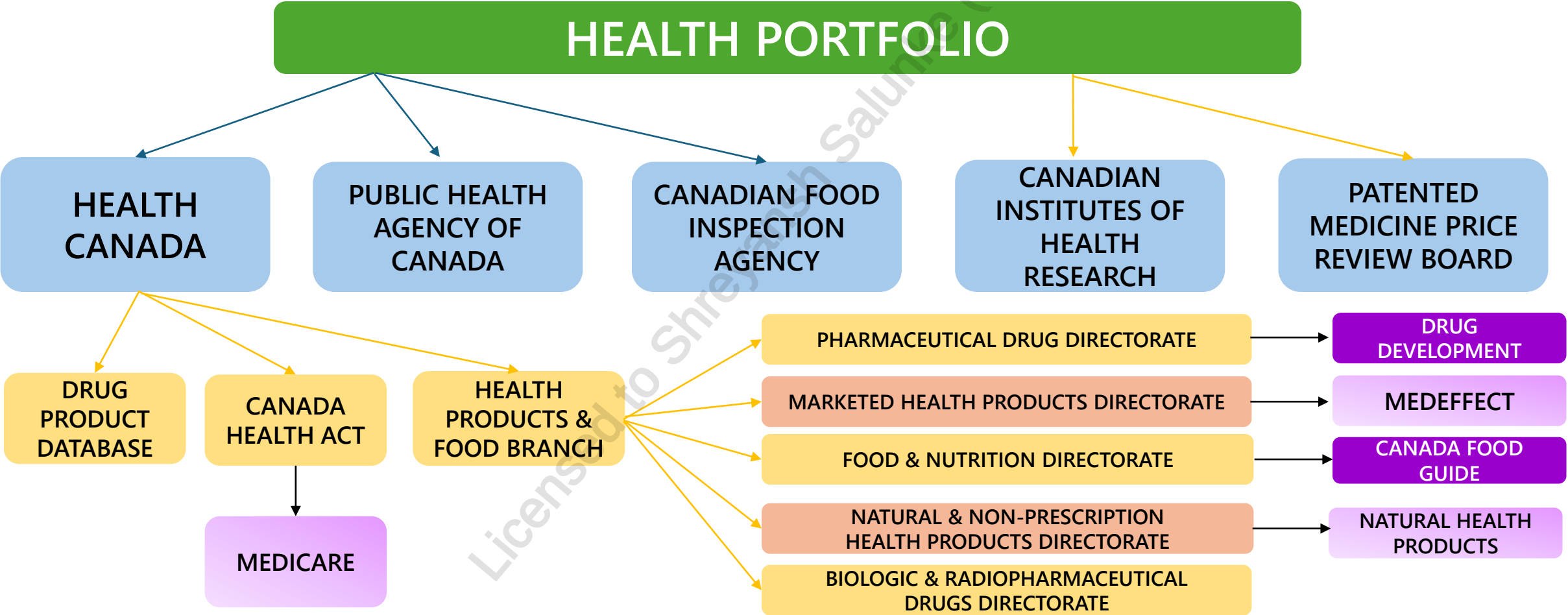


DRUG PRODUCT DATABASE (DPD)

- Provides access to information on ALL marketed, approved, and discontinued drugs
- Drug product monographs can be accessed via the DPD
- Does NOT provide medical advice, treatment options, pharmacological treatment algorithms or information on illnesses
- Managed by Health Canada
- Only complete list of marketed drugs available (Note: CPS / RxTx is not comprehensive)

HEALTH PORTFOLIO

MINISTRY OF HEALTH



HEALTH PRODUCT ADVERTISEMENTS - REGULATIONS

Health Canada

Health Canada is the national regulatory authority for health product advertisements.

- Establishes guidelines and policies to effectively regulate marketed health products,
- Writes the Food and Drug Act Regulations
- Oversees regulated advertising activities
- Strongly encourages drug companies to get their ads pre-cleared by Independent Preclearance Agencies
- Issues Terms of Market Authorization (TMA) which is comprised of all labelling information (e.g. product monograph, prescribing information, inserts, etc.) that accompanies the NOC and/or in the document that assigns a DIN, NPN or DIN-HM and any related labelling material for health products.

Note: Manufacturers are not legally required to submit ads to Health Canada for preclearance. However, most do so voluntarily to avoid costly rework, potential fines, or triggering stricter regulation.

HEALTH PRODUCT ADVERTISEMENTS - REGULATIONS

Independent Advertising Preclearance Agencies (APAs)

- Use Health Canada's guidance documents as well as their own codes of advertising to ensure advertising material submitted to them, complies with the F&DA, CDSA, and respective Regulations
- Ensure that the advertising is accurate, balanced and evidence-based, and does not expand upon or conflict with the TMA
- Ensures ads are NOT false or misleading
- Approved ads receive authorization to use the APA logo or an approval number (meaning it complies with the applicable legislation, regulations and guidance documents)
- Only products approved for sale in Canada can be advertised
- If an ad is NOT in compliance with F&DA or poses risk to public, Health Canada will issue regulatory letters instructing manufacturers to discontinue/correct the ad

TYPES OF ADVERTISING

1. Direct-to-Consumer Advertising (DTCA)

- Make therapeutic claims for a specific drug or brand
- Generally **illegal** in Canada with some exceptions made for vaccines (e.g. public health campaigns)

2. Reminder Ads

- Advertise the name (brand and chemical), price and quantity of drug directly to consumers without making any therapeutic indication (including innuendo or visual cues)
- **Legal** for schedule F drugs
- Example: "Viagra®. Talk to your doctor"

3. Help-seeking Ads

- Describe a disease or condition but do not recommend or suggest specific drugs
- Example: "Runny nose? You may be suffering from allergies. Talk to your doctor."
- **Legal**

INDUSTRY AND AGENCIES

Innovative Medicines Canada

Formerly Canada's research-based pharmaceutical companies (Rx&D)

- Association helping members to discover, develop, and deliver medicines and vaccines
- Ensure member compliance with a Code of Ethical Practices to ensure ethical interactions between members: government, private payers, stakeholders, and practitioners (e.g. role of pharmaceutical company in development of CEs/CMEs courses)
- <http://innovativemedicines.ca/>

Canadian Generic Pharmaceutical Association (CGPA)

- Represents manufacturers of generic drugs and active pharmaceutical chemicals
- Plays role in providing more affordable medications than brand-name medications
- <https://canadiangenerics.ca/>

Food, Health & Consumer Products of Canada (FHCP)

- Association representing manufacturers of evidence-based OTCs and NHPs with a goal of providing safe and effective self-care products
- <https://www.fhcp.ca/en/Policy-Regulation>

CANADA'S DRUG AGENCY (CDA)

Independent not-for-profit organization funded by Federal and P/T governments of Canada (except Quebec) that provides recommendations and evidence regarding correct use of drugs and devices to support healthcare professionals to make informed clinical decisions

Health Technology Assessment

- Evaluates implications of health technologies on patient health and the health care system (evaluates clinical, social, ethical, and legal effects)
- Examples of health technologies: diagnostic tests, surgical/medical devices and procedures

Drug Reimbursement Recommendations: Common Drug Review (CDR), pan-Canadian Oncology Drug Review (pCODR)

- After drug approval in Canada, Federal and P/T public plans must decide whether the drug will be eligible for public reimbursement
- CDA provides drug reimbursement recommendations and advice to federal and P/T public drug plans

VOLUNTARY PROFESSIONAL ASSOCIATIONS AND ADVOCACY

- National level: Canadian Pharmacists Association (CPhA)
- Provincial level: Ontario Pharmacists Association (OPA) etc.
- Practice-specific: Canadian Society of Hospital Pharmacists (CSHP)

Role:

- Advocating for, and representing the views of pharmacists and pharmacy practice
- Negotiating fees and agreements with governments
- Providing continuing education and professional development
- Publishing journals and providing drug information, for example: CPhA publishes CTC (RxTx), CPS

REGULATORY COLLEGES

Regulate profession and ensure patient safety

1. Licensing
2. Regulations
3. Enforcement
4. Discipline



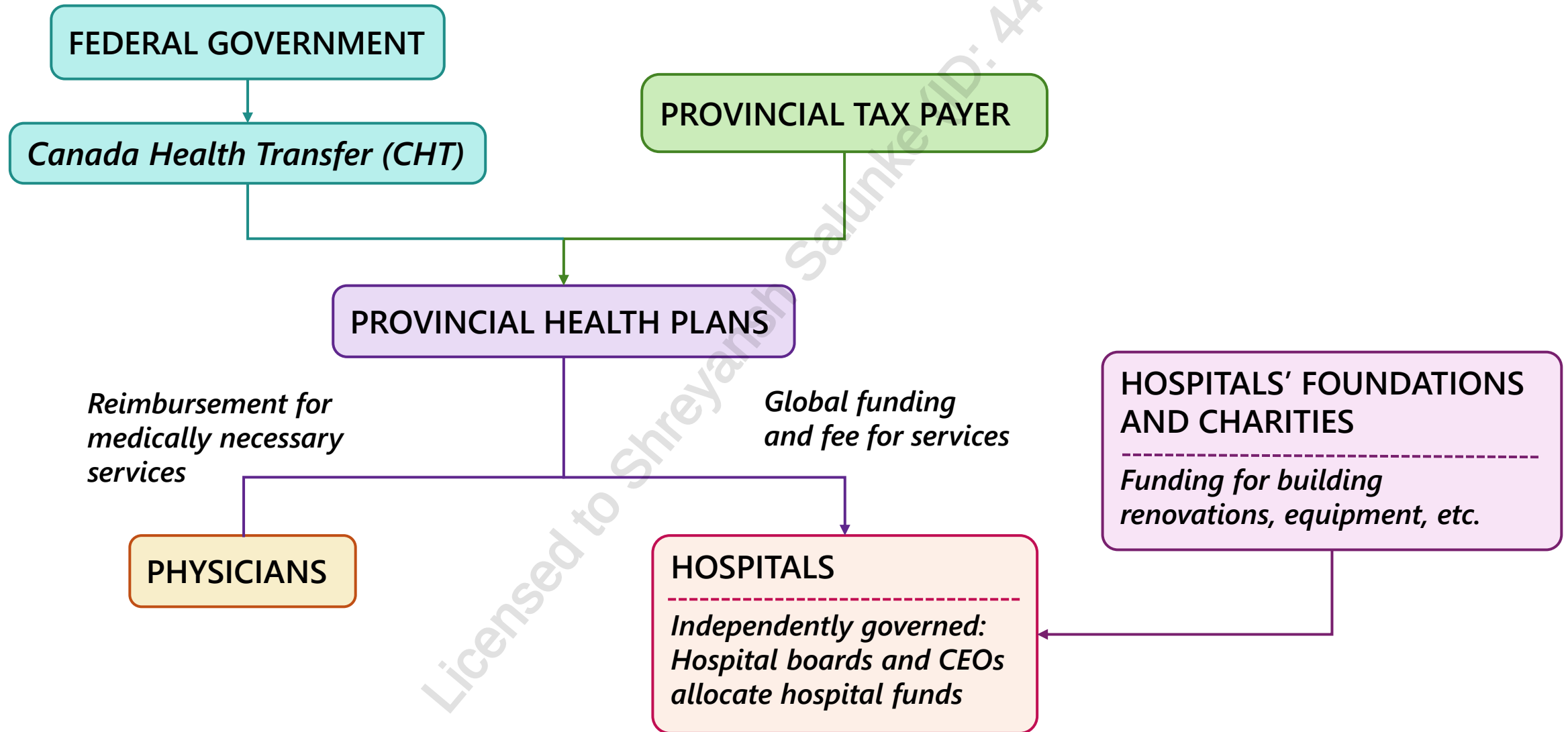
Ontario College
of Pharmacists

Putting patients first since 1871



Alberta
College of
Pharmacists

HOSPITAL FUNDING



HEALTH CARE SYSTEM FUNDING

Provincial Ministries of Health

- Administers health services based on federal funding
- Provides additional funding for health services (province specific)

Regional Health Boards

- Example: Ontario Health Agency
- Planning and allocating resources to local health services

Hospital Administrators and Hospital Boards

- Have power at the local level about services provided and allocation of funding

Hospital Foundations and Charities

- Often fund specific purchases (e.g. buildings, renovations, equipment) or endowments

HOSPITALS

Semi-private

- Organizations in charge of the hospital make decisions regarding day-to-day allocation of financial and human resources (*private*)
- Hospitals are often supported by a non-profit, non-governmental, charitable foundation
- However, the government provides strict authority over hospital budgets/large financial transfers (*public*)
- Hospitals provide a broad range of services, including acute emergency care; diagnostic testing; prescription drug therapy; rehabilitation services; counselling; and palliative care

COMMUNITY PRACTICE

REIMBURSEMENT FOR CLINICAL PHARMACY SERVICES

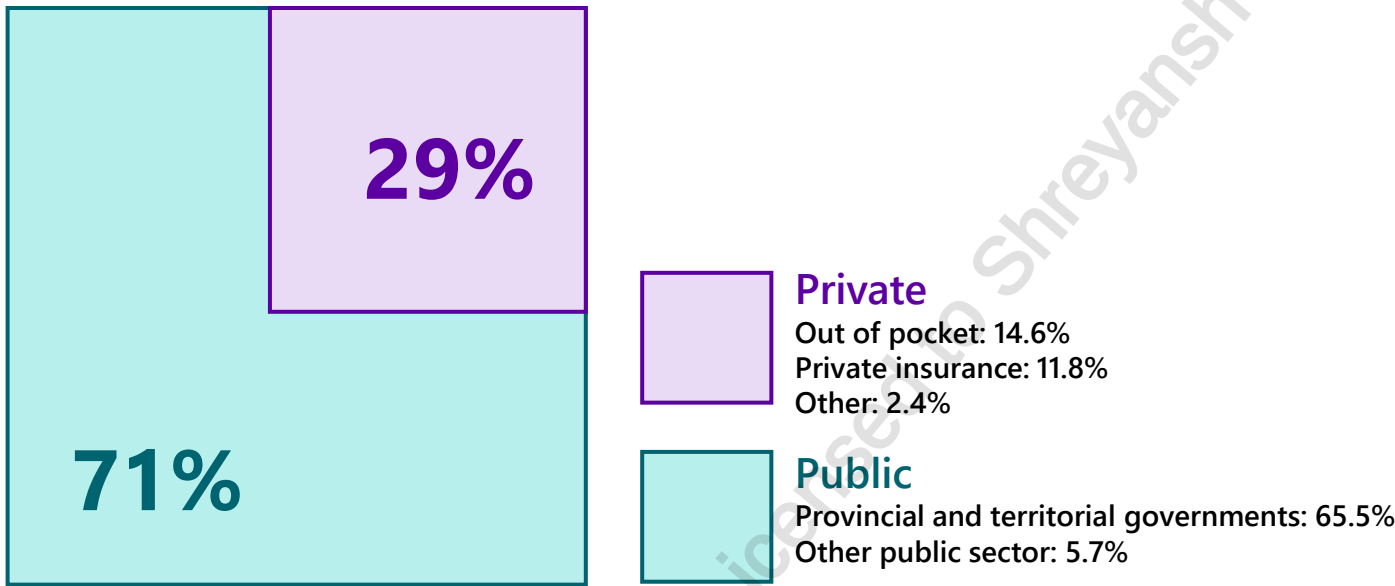


HEALTH EXPENDITURES AND TRENDS

Total Health Care Spending Annual Growth Rates:

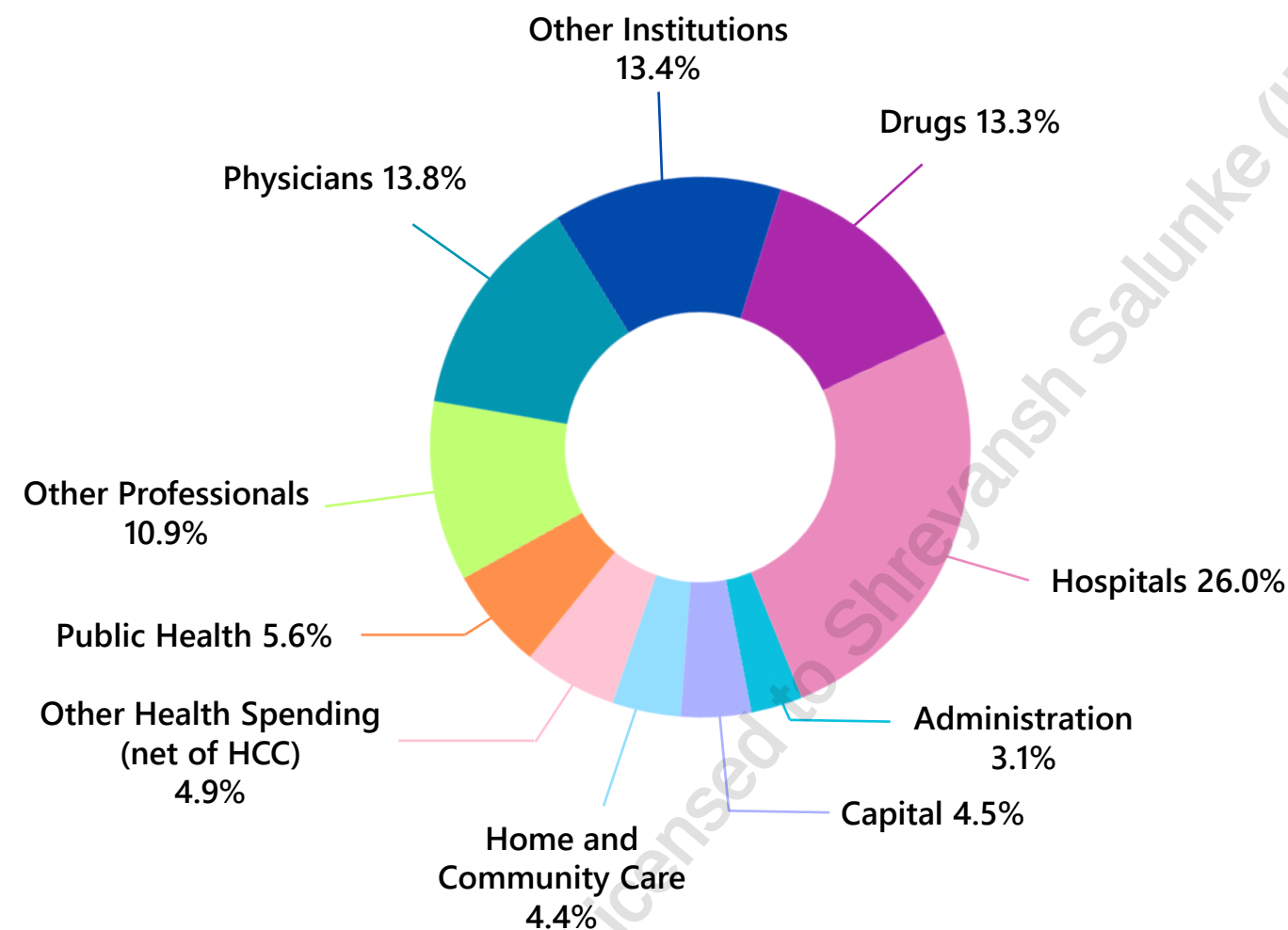
2008-2010	2011-2019	2020	2021	2022	2023	2024	2025 (forecast)
5.9%	3.9%	12.9%	8.3%	1.8%	7.4%	6.1%	4.2%

Total Health Expenditure By Source Of Finance (2024 Forecast):



Note: Numbers may not add up to the total due to rounding
Source: National Health Expenditure Database, Canadian Institute for Health Information

TOTAL HEALTH EXPENDITURE BY USE OF FUNDS

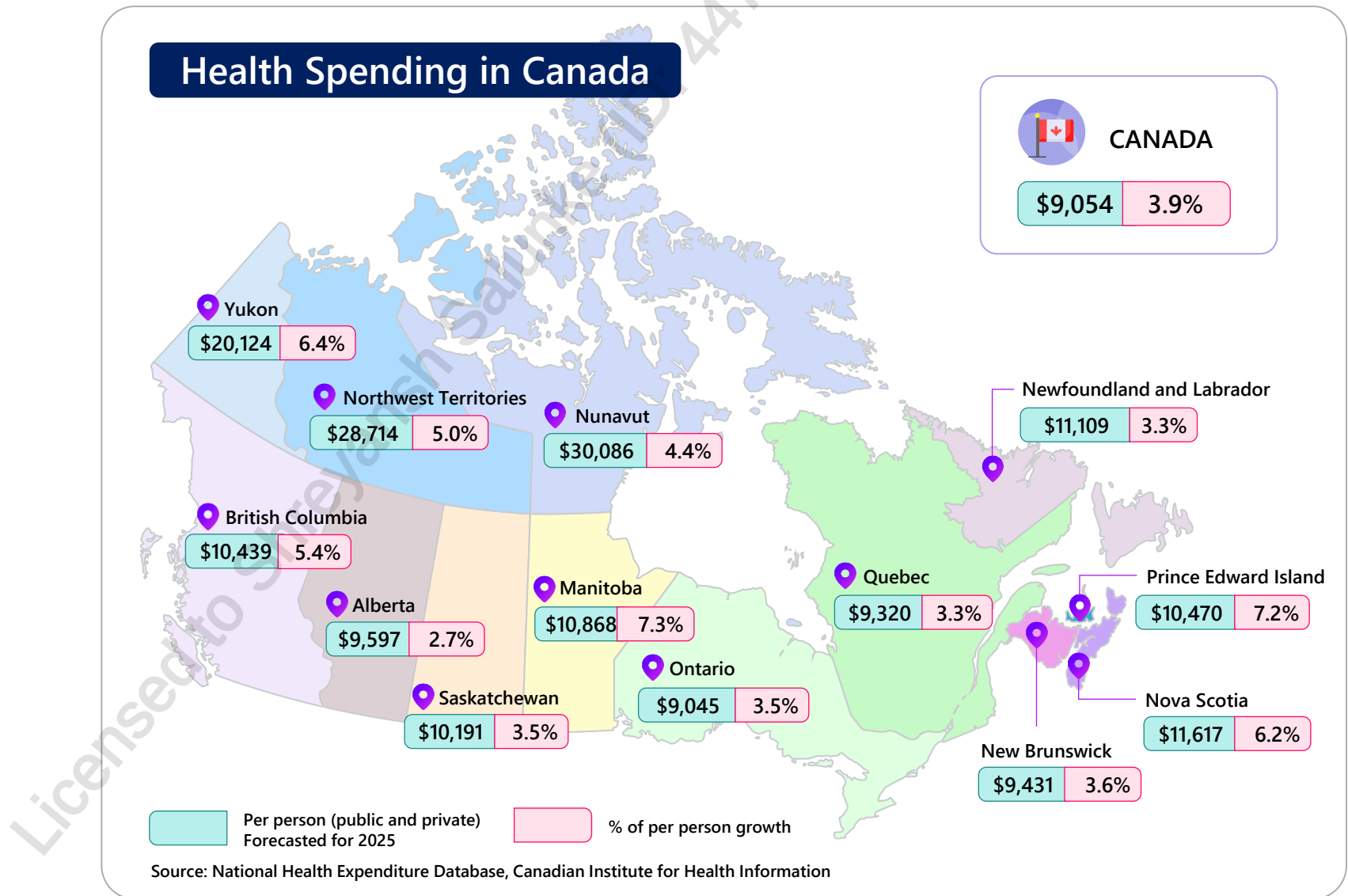


Largest share of health spending (~50% total health spending): hospitals, drugs, and physician services

Source: National Health Expenditure Database, Canadian Institute for Health Information

HEALTH EXPENDITURES IN PROVINCES AND TERRITORIES

Health expenditures can vary across P/T due to differences in the services provided in each, demographic factors, population density, etc.



HEALTH LITERACY

“Ability to access, comprehend, evaluate, and communicate information as a way to promote, maintain, and improve health in a variety of settings across the life-course” - PHAC

Risk factors for ↓ health literacy

- > 65 years of age
- Minority populations
- Living without medical insurance
- Low socio-economic status
- Poor health status
- Low degree of education

Consequences of ↓ health literacy: poor health outcomes and increased healthcare costs

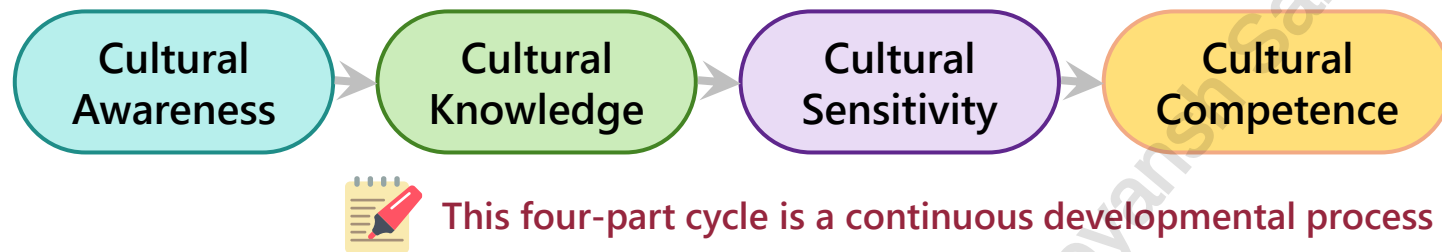
How to ↑ health literacy

- Speak clearly and concisely
- Do NOT use any medical jargon
- Encourage patients to ask questions
- Provide visual aids
- Provide concise written information
- Encourage patients to repeat information back to you

CULTURAL COMPETENCY AND DIVERSITY

Attitudes, knowledge, skills and values an individual has and uses when working in cross-cultural environments

Cultural Competence Model™



Becoming culturally competent

- Awareness of diversity and ability to be non-judgmental
- Ability to navigate cultural dynamics, as actions (e.g. silence) may have different meanings across cultures

FIRST NATION, INUIT AND ABORIGINAL POPULATIONS

Indigenous Peoples health status:

- Shorter life expectancy, higher infant mortality, higher suicide rate
- Greater prevalence of chronic diseases

Federal government provides services to addiction treatment centers, mental health services, community care workers, nursing stations, homecare services

- Services may be limited or inconsistently available

Health Canada's First Nations and Inuit Health Branch (FNIHB) works with other jurisdictions to promote wellness and disease prevention

Overall health status of Indigenous Peoples has improved, but gaps remain in the overall health status when it is compared to other Canadians

TAKEAWAY

- **Canadian Health Care System** is funded federally (Health Canada Transfer), provincially (taxpayer) and privately
- Five key principles of the Canada Health Act include **Public Administration, Comprehensiveness, Universality, Accessibility and Portability**
- Minister of Health oversees the **Health portfolio** comprising of Health Canada, PMPRB, PHAC, CIHR and CFIA
- **PDD** regulates prescription drugs for human use in Canada
- Clinical Trials for drug development require approval by the HPFB before beginning
- A **NDS** must be filed for approval following positive clinical trials before a drug can be sold in Canada
- Canada Vigilance Program is a part of **Medeffect** and a useful post-marketing surveillance tool
- Pre-clearance by Health Canada for health product advertisements is **voluntary**
- Three levels of patient care – **primary, secondary and tertiary**
- Four categories of patient care – **inpatient, outpatient, ambulatory and telehealth**

PRACTICE CASE

1. A drug company recently completed their Phase III clinical trial and would now like to submit a New Drug Submission (NDS) for final approval. Once they have completed their NDS where should this be submitted?
 - a) Health Canada
 - b) Public Health Agency of Canada
 - c) Patented Medicine Price Review Board
 - d) Health Products & Food Branch

2. The drug company has received approval and their new drug can be marketed in Canada. They would like to verify if their sale price is appropriate. Where is the MOST appropriate place to get this information?
 - a) Ministry of Health
 - b) Pharmaceutical Drug Directorate
 - c) Patented Medicine Price Review Board
 - d) Public Health Agency of Canada

PRACTICE CASE

3. If the drug company would like to advertise this product, it is mandatory for them to use an Independent Pre-clearance Agency?
- a) True
 - b) False
4. What type of advertisement is MOST appropriate if this is a pharmaceutical drug product?
- a) Help-seeking ad
 - b) Direct-to-consumer ad
 - c) Reminder ad
 - d) Health promotion ad

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CHANGE LOG

March 2025

- Slide 2: Updated legend
- Slide 39: Removed CADTH from title, kept CDA. Also changed CADTH to CDA on slide
- Slide 42: Added missing arrow from provincial health plan to physicians

April 2025

- Slide 9: Removed medical devices from Pharmaceutical Drugs Directorate

August 2025

- Formatting and rephrasing edits made throughout
- Graphic updates made throughout
- Updated 'Office of Nutrition policy and promotion' to 'Food and Nutrition Directorate'
- Slide 27: Changed to 'Traditional medicines (e.g. Chinese medicines)'
- Slide 29: Replaced 'emergency departments' with 'community health centres, nursing stations', updated secondary care information
- Slide 33: Added information to DPD
- Slide 45: Replaced link
- Slides 47 & 48: Updated statistics

November 2025

- Slide 11: Updated the purpose of Phase III clinical trials, added Phase IV
- Slide 16: Added point about scientific review
- Slide 36: Added the word "Advertising" to APA's
- Slide 55: Updated references

January 2026

- Slide 2: Updated legend
- Slide 23: Added new slide on Canada Dental Care Plan

February 2026

- Slide 2: Updated NAPRA competencies
- Slide 20: Added note about bill C-64
- Slide 23: Added covered services and marked section FYI
- Slides 47/48/49: Updated values to reflect Nov 2025 data release
- Slide 58: Updated references